

Neurosurgery **For use on Ward (Not NCCU)**
Emergency Subarachnoid Haemorrhage (SAH)
Clipping or Coiling of Cerebral Aneurysm – Pathway Summary

**Optimum
LOS:
6-8 days**

Ward:
Admitted: DD / MM / YYYY
EDD: DD / MM / YYYY

For staff use only:
Hospital no:
Surname:
First names:
Date of birth:
NHS No: ____ / ____ / ____
(Use hospital identification label)

This document is a summary of care, it includes the **crucial components of care, but **not** all.
To be completed by all staff caring for the patient, initial the boxes, where present when tasks completed/ achieved. If tasks are not completed record the reason for exception in the patient's notes.*

	Day 1: Admission / Pre-op DD / MM / YYYY	Post-Operation DD / MM / YYYY	On-going Care: Days 2-6 DD / MM / YYYY	Day 6-8: Discharge DD / MM / YYYY
Date Consultant Bleep				
Nursing	Neuro obs & vital signs as per protocol & NEWS (increase frequency if patient condition indicates) bedrest (no head elevation restrictions) out to commode only Daily Strict fluid balance (input / output) (3 litres input - care to be taken with those who are elderly or have a cardiac history) Patient Information given to patient and family (contact number for CNS)	Op notes read re: post-op instructions Neuro obs & vital signs as per protocol Observe for stroke like symptoms and report urgently (vasospasm) Observe for hypo / hypernatraemia IVI use N.Saline only. NEVER use dextrose saline or gelofusin on ward Sit out of bed in appropriate chair, Mobilise as able	Decrease frequency of observations when GCS stable continue strict fluid balance (input / output) Clips / sutures removed on day 5 TTO POD check completed / sent to pharmacy Confirm if patient has 7 days medication (at home or locker). If NOT order via TTO	Discharge by nurse if all care completed according to pathway (see medical notes for criteria or instructions) Nursing discharge checklist completed Ensure patient has transport in place TTO's given & explained Give discharge information, wound leaflet & relevant contact details (eg CNS)
Medical	- Daily Bloods: U&E, FBC - Daily Input – up to 3 litres / 24hrs daily. N.Saline 0.9only - Review antihypertensives BP should be below 180mmHg until treated. ECG - Anti-embolic stockings - Anti-seizure if indicated - Analgesics - antiemetics - Nimodopine 60mg 4hrly or 30mg 2 hrly if BP <90 (21 day course) - Clip/coil within 72 hours of ictus (if clinically well)	- Op/procedure notes read re: post-op instructions i.e. antiplatelet therapy for endovascular treatment - Management of hydrocephalus EVD if blood in 3rd/4th ventricles – lumbar drain if no blood in 3rd/4th ventricles - Prescribe LMWH (24hrs post -op) Strict daily bloods	Strict daily bloods	Do not discharge if headache severe (vasospasm risk) Discharge by consultant if exceptions to pathway [n/a] Discharge letter finalised by 11am Clinic 3 months
AHP		Physio to assess mobility OT for cognitive assessment SALT if required	Physio to progress mobility as able	Stair assessment if indicated OT assessment if indicated